

HTR Book — Copy-Paste Markdown Templates

Paste these patterns into the manuscript Markdown to get the styled treatments.
The build (build_docx.py) renders them automatically. Keep the syntax **exact** — the post-pass matches on it.

Stat strip (headline numbers → navy cards)

Each stat on its own line as **VALUE** – Label, separated by blank lines.

```
::: {custom-style="StatStrip"}
**$2.4B** – 5-Year Deficit Projection

**108%** – Premium Increase 2018–2024

**9 of 14** – Hospitals in Loss FY2023

**57%** – Growth in 65+ by 2040
:::
```

Callout boxes

Label line is bold; body follows. Boxes may contain bullet lists.

```
::: {custom-style="CalloutBeyond"}
**BEYOND VERMONT**

The lesson generalizes: any state that layers payment reform onto missing data infrastructure gets the same cascade.
:::

::: {custom-style="CalloutWorked"}
**WORKED EXAMPLE – Hospital CFO under a voluntary ACO**

Imagine you are the CFO of a Vermont hospital generating $40M from inpatient admissions...
:::

::: {custom-style="CalloutTry"}
**TRY THIS – Reproduce the OneCare cascade.**

Open the HTR Simulator (/htr-simulator`) and build a profile with strong Economics but weak Technology...
:::

::: {custom-style="CalloutKey"}
**KEY NUMBERS**

**5.8:1** – Blueprint ROI

**91%** – Primary care access
:::

::: {custom-style="CalloutVT"}
**VERMONT IN PRACTICE – The $400M Savings Estimate**

Oliver Wyman projected more than $400M in direct savings over five years...

– Close unsustainable inpatient facilities
– Reduce administrative costs
:::

Variants of CalloutVT (just change the label text): VERMONT EVIDENCE, GO DEEPER – HTR Academy.
```

Banner (one punchy thesis line)

```
::: {custom-style="Banner"}
Vermont is not an outlier. It is a preview.
:::
```

Pull-quote + attribution

```
::: {custom-style="PullQuote"}
"If the model is not adequately regulated and sufficiently resourced, it will fail dramatically."
:::
::: {custom-style="QuoteAttr"}
– Owen Foster, Chair, Green Mountain Care Board, January 2025
:::
```

Epigraph (section opener)

```
::: {custom-style="Epigraph"}
You cannot manage what you cannot measure.
:::
```

Tables (auto-styled: navy header, full width, content-fit columns)

Use normal Markdown pipe tables. Always add a caption line right after:

```
| Pillar | Diagnostic question | Structural role |
| :--- | :--- | :--- |
| Policy | Is it permissible? | The mandatory architecture. |

*Table X.Y – Caption text. Source: ...*
```

For tables whose cells need **bullet lists**, use a Pandoc **grid table** (see the Layer tables in the manuscript for the exact form) — pipe tables can't hold bullets.

Key Concepts (auto-styled — just use bold term + definition)

Key Concepts in This Chapter

Dependency logic
The principle that some pillar investments cannot produce results until upstream investments are in place.

Execution sequence
The order in which the six pillars must be built: Policy, Technology, Economics, Clinical, Equity, Operations.

(The build renders the term blue-bold-10pt and the definition 9pt automatically.)

Sources (auto-styled small italic)

Sources

Vermont Agency of Human Services, Health Care System Transformation Report (November 2025); Oliver Wyman, Act 167 Recon

Platform + Academy tie-ins (the book's selling point — use generously)

- Link a tool by its real path (verify against frontend/lib/taxonomy/tools.ts):
 HTR Simulator (`\\htr-simulator`), **FHIR Lab** (`/research-lab/interoperability?tab=fhir`).
- Point readers to a course with a CalloutVT "GO DEEPER — HTR Academy" box naming the course (e.g. Value-Based Care, Population Health, Health Equity).

Sequence/diagram graphics

Reference an image generated into book-build/:

![Alt text describing the figure.](sequence_graphic.png)

Figure X.Y – Caption.

Generate/edit the graphic in make_sequence_graphic.py (Pillow).